

Two molecules approved.

Nine in the race.

GLP-1 medicines reshaped the treatment of type 2 diabetes, then of obesity.

What follows is the landscape by molecule: mechanism, stage and indication.

No share prices. No companies. Just the molecules and where they stand.

US prescriptions for GLP-1 receptor agonists rose 587% between 2019 and 2024.

Sources listed on the final slide. As at August 2026.

APPROVED FOR OBESITY

Only two mechanisms are on the market.

semaglutide

Approved

GLP-1 receptor agonist · injectable and oral

tirzepatide

Approved

dual GIP/GLP-1 agonist · injectable

One activates the GLP-1 receptor alone.
The other adds GIP, a second incretin receptor.

Everything else in obesity is still in trials.

That is unusual for a therapy area this large, and it
is why each trial readout moves the field so much.

Obesity prevalence is projected to exceed 50% of adults by 2030.

The next contest is oral.

oral semaglutide 25 mg

Approved

peptide in tablet form

Authorised in the EU in July 2026 — the first GLP-1 receptor agonist in tablet form for weight management there, and its fifth authorisation after the US, UK, UAE and Bahrain.

About 17% weight loss versus 3% on placebo in Phase 3b.

aleniglipron

Phase 3

oral GLP-1, small molecule

A small molecule rather than a peptide, which is simpler to manufacture at scale. Phase 2b showed up to 16.2% weight loss in the extension study, with a 10.4% discontinuation rate.

A tablet changes who can be treated, not just how well.

BEYOND GLP-1 ALONE

Amylin is the second mechanism.

cagrilintide + semaglutide

Filed

GLP-1 + amylin · injectable

zenagamtide

Phase 3

GLP-1 + amylin receptor agonist

petrelintide

Phase 2

amylin analogue

ACCG-2671

Phase 1

oral amylin, small molecule

If approved, the first combination would pair a GLP-1 receptor agonist with an amylin analogue for the first time. A decision is expected in Q4.

Amylin targets satiety by a different route, so the two mechanisms may add rather than overlap.

The same biology is moving into the liver.

semaglutide

GLP-1 receptor agonist

Approved in MASH

One of only two treatments approved for MASH.
The other works on a thyroid hormone receptor.

pemvidutide

balanced 1:1 glucagon/GLP-1 agonist

Phase 3

Adding glucagon acts directly on the liver, while GLP-1 handles appetite and weight. In Phase 3 for MASH from August 2026: about 1,790 patients, 52-week data expected in 2029.

Also in development for alcohol use disorder and alcohol-associated liver disease, with FDA Fast Track in both, and Breakthrough Therapy status in MASH.

Metabolic, hepatic and addiction pathways converge on one receptor family.

What this covers.

Approval status, mechanism and trial stage as at August 2026, taken from company filings, company releases and industry landscape reviews.

No trade names are used. No share prices, valuations or company comparisons appear here — this is a view of the science, not of the market.

Trial results are point-in-time and stages change. Nothing here is medical or investment advice.

Sources

SEC filings (10-Q, 8-K) and company releases, Dec 2025 – Aug 2026; regulatory announcements; industry landscape reviews.

Related work

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